

THE CROSSING TIME

BY PORTA VOZ DO IMIGRANTE

EDITORIAL GUIDE · MENTAL HEALTH IN THE CROSSING

The Immigrant's Handbook: Without Depression

*How to recognize the signs, where to find help in France, and
what to do, in practice, to protect your emotional health far
from home.*

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BY PORTA VOZ DO IMIGRANTE

The crossing also happens within

No one emigrates only from a country. You emigrate from a language, from a web of affection, from a way of existing. And that second crossing, the invisible one, is the one that weighs the most.

We speak naturally about visas, housing, work, and paperwork. We speak far less about what happens when the euphoria of arrival fades and the routine in another language starts to collect its price. The distance from family, the bureaucracy that never ends, the cold, the feeling of starting from zero: none of this is weakness. It is the real experience of millions of people who live between two countries.

This handbook was made for the Brazilian community in France, but it serves anyone living through a crossing. It does not replace a doctor, a psychologist, or an emergency service. It does something else: it organizes the information no one hands you on arrival, shows in numbers what France itself is going through, points to the concrete paths to help within the French system, and gathers practices of emotional protection, starting with the most forgotten one: keeping your family informed about your life here.

Information that positions you. About mental health, too.

ANNE LOPES · EDITORIAL DIRECTION

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CHAPTER 01

What immigration does to the mind

The part of the crossing no one photographs.

What immigration does to the mind

Moving countries is one of the highest-impact emotional events an adult can live through. The psychology of migration describes a path that repeats itself, with variations, in almost every trajectory. Knowing that path changes everything: what looked like a personal flaw turns out to be a predictable phase.

The four phases of adaptation

1. **Honeymoon.** Everything is new: the city, the language, the bakery on the corner. The adrenaline of arrival masks the tiredness and the homesickness. It can last weeks or months.
2. **Culture shock.** Novelty becomes obstacle. Bureaucracy stalls, the language exhausts, social codes confuse, winter goes dark at 5 pm. It is the phase when many ask themselves whether they made the right decision. It is also the phase when the first symptoms of emotional distress usually appear.
3. **Adjustment.** Little by little, daily life gains logic. You understand the system, make your first local connections, solve alone what used to require help. The emotional swings continue, but with shallower dips.
4. **Adaptation.** The country stops being scenery and becomes life. That does not mean forgetting Brazil: it means managing to exist whole in both places.

WHAT THIS CHANGES

Knowing that culture shock is a phase, not a sentence, takes away half its power. Those who understand the path ask for help earlier, blame themselves less, and cross better. And be aware: the phases are not linear. A layoff, a loss grieved from afar, or a titre de séjour renewal can push someone already adapted back into shock. Falling back a phase is not personal regression: it is life testing the structure, and it is exactly in those moments that the practices in chapter 06 matter most.

Migratory grief

Every immigrant lives through a grief, even when the move was chosen and wanted. It is the grief for the language, for the family close by, for childhood friends, for the professional status left behind, for the landscape, even for one's own name pronounced correctly.

Unlike other griefs, this one is recurrent: it reopens with every video call, every missed family celebration, every December. And it is a grief without social recognition. Those who stayed in Brazil see the Eiffel Tower; those who crossed carry the emotional bill in silence.

When that grief accumulates without space to process it, added to chronic stressors such as precarious paperwork, isolation, and work overload, the ground for anxiety and depression is ready. The clinical literature describes chronic stress conditions linked to migration in extreme situations of loneliness, fear, and lack of resources.

The point that matters here is a single one: psychological suffering in immigration is a documented, frequent, and treatable phenomenon. It is not a character flaw, not ingratitude, not weakness.

What helps to process this grief

- **Name it.** Calling what you feel grief, not fussiness, changes how you treat it.
- **Tell the story.** Talking about the life left behind, with someone who truly listens, is processing, not unproductive nostalgia.
- **Keep rituals.** Coffee the way it is made back home, the music, the Sunday recipe: bridges, not crutches.
- **Accept double belonging.** You do not have to choose between being from here or from there. Health lies in being from both.

The crossing defines who you become.

And no crossing needs to be made without support. The next chapters show how to recognize the moment to ask for help, and exactly where to find it in France.



CHAPTER 02

Warning signs

Suffering rarely announces itself. It settles in.

Warning signs: when sadness stops being temporary

Missing home, crying after hanging up a call with your mother, having bad days in winter: all of this is part of the migratory experience. The alert lights up when the suffering changes nature: it stops being a reaction to a moment and becomes a permanent state that swallows the routine.

Signs in you

- Sadness, emptiness, or irritability present almost every day, for more than two weeks in a row.
- Loss of interest in what used to bring pleasure: cooking, going out, talking to friends, planning the future.
- Persistently disrupted sleep: insomnia, or wanting to sleep all day.
- Appetite and weight changing without explanation.
- Constant tiredness, the feeling that any task demands an enormous effort.
- Difficulty concentrating at work, in studies, even on a TV series.
- Disproportionate guilt, a sense of failure, thoughts like "I should not have come", "I am a burden", "I am useless".
- Growing isolation: declining invitations, avoiding family calls, disappearing from the groups.
- Increased use of alcohol or other substances to "switch off".
- Thoughts of death or a wish to disappear, at any intensity.

ATTENTION

None of these signs, on its own, closes a diagnosis. Several of them together, for weeks, indicate it is time to seek professional assessment. And one of them changes the urgency of everything: if there are thoughts of death or of self-harm, help must be sought immediately. In France, call 3114, the national prevention line, free of charge, 24 hours a day. In an emergency, 15.

Signs in someone close

In immigration, it is often the friend, the coworker, or someone from the church group who notices first. The person who is suffering rarely announces it: they vanish, change, minimize. Pay attention to someone who:

- Disappeared from gatherings and social networks without explanation.
- Visibly changed in appearance, mood, or rhythm.
- Often says they are "tired of everything", that "nothing is worth it", or that they "wish they could disappear".
- Started giving away important belongings or saying goodbye in a strange way.
- Lives a situation of stacked risk: job loss, breakup, expired papers, debts, recent bereavement.

Chapter 07 shows what to do in these cases, sentence by sentence. The general rule is simple: asking directly, calmly and without judgment, does not make things worse. Silence does.

Passing sadness or warning sign?

Sadness of the crossing: it comes in waves, tied to clear triggers (dates, calls, news), and coexists with good moments.	Warning sign: constant, without pause, for more than two weeks, and the good moments have disappeared.
You vent, you cry, and then you manage to carry on with the day.	Talking no longer brings relief, or you have stopped talking to everyone.
The routine still stands: work, home, contacts.	The routine has started to crumble: absences, unpaid bills, isolation.

WHY IMMIGRANTS TAKE SO LONG TO ASK FOR HELP

Shame at having "failed" the migration project, fear of worrying the family, the language barrier, not knowing the French system and, for those without papers, fear of approaching any public service. Each of these barriers has an answer in this handbook: chapters 04 and 05 dismantle the last three, and the first two lose their force once the subject stops being taboo in the community.

A man in a dark coat and trousers walks down a wet, reflective city street. A large, billowing, grey cloud or smoke cloud is positioned directly above his head, partially obscuring it. Other pedestrians are visible in the background, some holding umbrellas, suggesting a rainy day. The overall mood is somber and atmospheric.

CHAPTER 03

The French picture in numbers

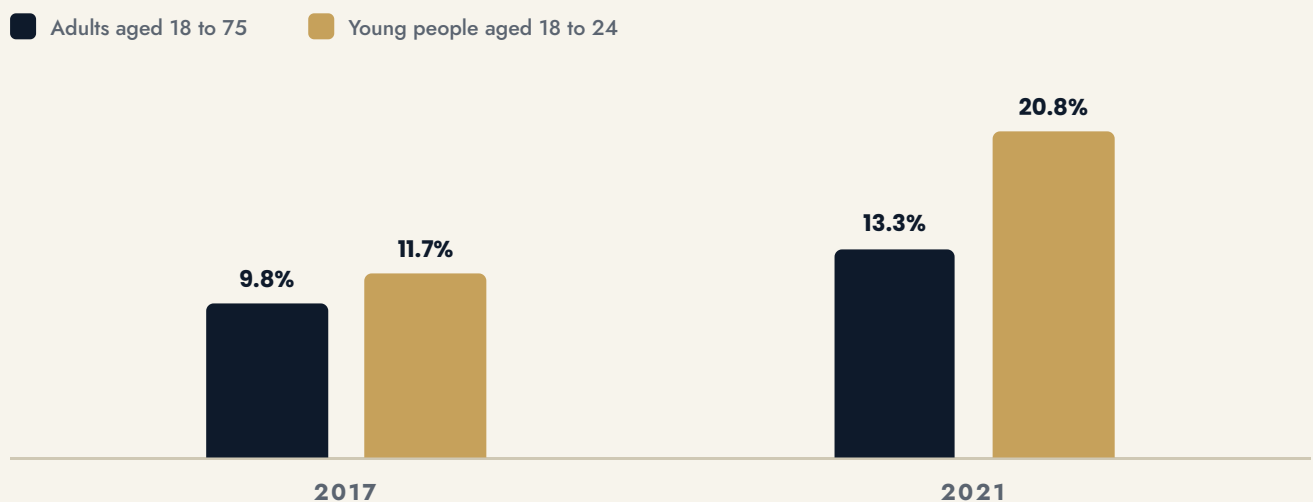
What France's official data reveals about the decade.

The French picture in numbers

This is not an individual problem, and official French data proves it. The numbers show clear growth in depression and suicidal behavior among the young. Suicide mortality in the total population, however, shows no continuous growth: it fell for decades, stabilized, and had a small recent rise.

Depression: general advance, surge among the young

Between 2017 and 2021, the annual prevalence of depressive episodes rose from 9.8% to 13.3% among adults aged 18 to 75. Among young people aged 18 to 24, it jumped from 11.7% to 20.8%: one in five.

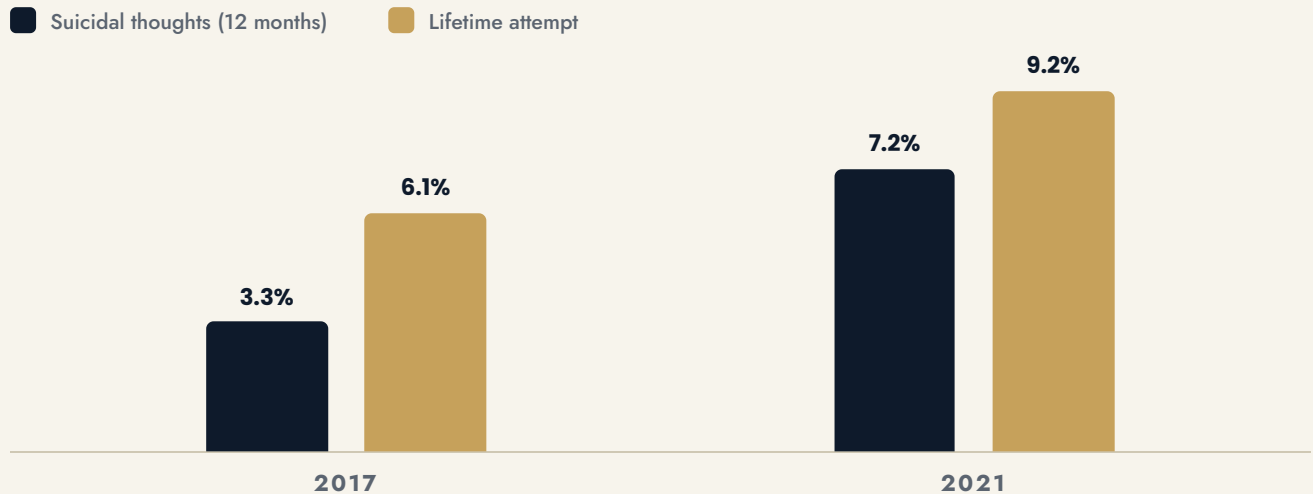


Percentage reporting a depressive episode in the previous 12 months. Source: Baromètre de Santé publique France.

For the immigrant community, the reading is direct: the host country is itself living under growing pressure on mental health, especially among the youngest. Those who arrive add to that reality the specific stressors of the crossing described in chapter 01.

Suicidal behavior among the young

Among French people aged 18 to 24, suicidal thoughts in the past year went from 3.3% to 7.2% between 2017 and 2021. The share reporting a suicide attempt at some point in life increased from 6.1% to 9.2%.



Reported prevalence among French people aged 18 to 24. Source: Santé publique France.

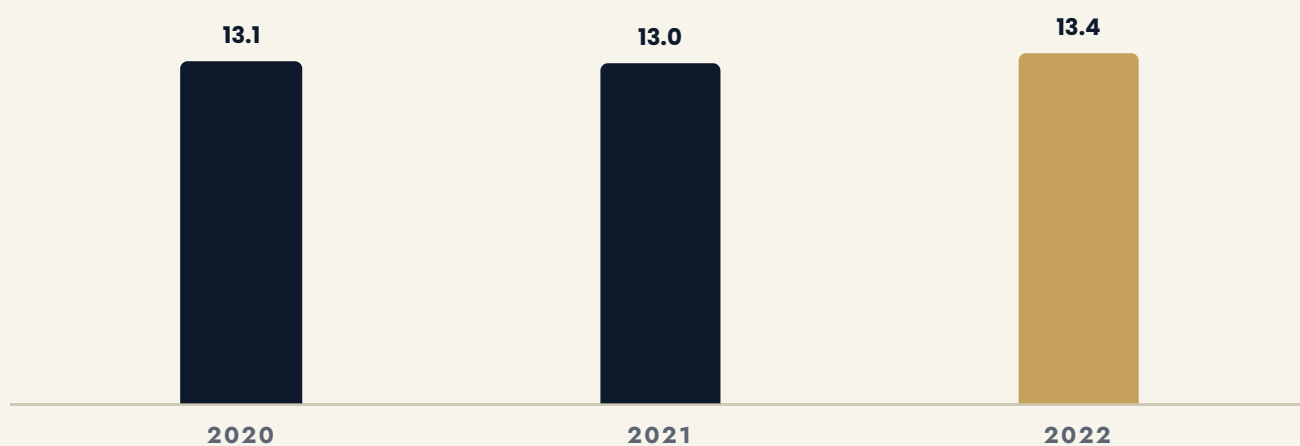
METHODOLOGICAL NOTE

The 2024 results should not be added directly to the earlier curves of suicidal behavior, because the French Baromètre changed its survey method. Valid comparisons stop in 2021.

These numbers explain why France created a dedicated national line (3114) and specific services for young people and students, presented in chapter 05. The public response exists; what is missing is for the information to reach those who need it.

Suicide mortality: the end of the historic decline

In the total population, the picture differs from what common sense suggests. Suicide mortality fell for decades in France. Between 2020 and 2021 it held stable, and in 2022 it rose slightly: around 9,200 deaths recorded in the year, or 13.4 per 100,000 inhabitants.



Suicide deaths per 100,000 inhabitants. Source: CépiDc/Inserm, DREES synthesis.

It is therefore more accurate to speak of the end of the historic downward trend and a slight recent rise, not of continuous growth. Precision matters: dramatizing the overall figure misinforms, but ignoring the youth curve would be worse. The decade's warning sign sits in the 18-24 age group, and it is toward that group that the services in the next block of chapters point most strongly.

Mobility is power. Caring for the mind is what sustains that power.

How to find help in France: the step by step

The French system has real entry doors for mental health, including free ones. The problem is that no one explains this to the immigrant on arrival. This chapter organizes the path, from the mild case to the emergency.

Step 1: the general practitioner (médecin traitant)

In France, the general practitioner is the entry door for almost everything, mental health included. They can assess the situation, rule out physical causes (thyroid, vitamin D deficiency, anemia), prescribe treatment when indicated, and refer you to a psychologist or a psychiatrist. If you have not yet declared a médecin traitant in your Ameli account, this is one of the most important steps of your French administrative life.

Step 2: a psychologist reimbursed through Mon soutien psy

The public program Mon soutien psy grants up to 12 psychologist sessions per calendar year, reimbursed by the Assurance Maladie, with participating psychologists, and appointments can be booked directly. The list of participating professionals and the updated rules are on ameli.fr and on the government website. Check the conditions in force at the time of booking, as the program has evolved in recent years.

Step 3: the public mental health centers (CMP)

The Centres Médico-Psychologiques are public multidisciplinary units: psychiatrists, psychologists, nursing, and social work, with consultations fully covered by the Assurance Maladie. Access follows the address where you live; the town hall or the médecin traitant will point you to the CMP of your sector. The wait can be long, so register early and keep the other fronts moving in parallel.

In Portuguese, and even without papers

Care in Portuguese

Speaking about pain in another language is a real barrier: no one should need a dictionary to say they are suffering. There are paths:

- Brazilian and Portuguese-speaking psychologists in France, in person in the Paris region or online. Look for registered professionals (in France, the title of psychologist is regulated and the ADELI or RPPS number can be verified).
- Online therapy with psychologists in Brazil, regulated by the Brazilian Federal Council of Psychology. For those starting their life in France, it is often the fastest route, with fees in Brazilian reais.
- Community associations: Brazilian groups, welcome networks, and churches often keep lists of Portuguese-speaking professionals and support groups. Ask. The information circulates, but it only reaches those who ask.

Without papers, without coverage: you still have rights

An irregular migration status does not cancel the right to care. Public hospitals maintain the PASS (Permanences d'Accès aux Soins de Santé), care access points for people without coverage. Those who have been in France for more than three months in an irregular situation can apply for the AME (Aide Médicale de l'État), which covers health care, mental health included. And the emergency room treats everyone, always, without exception and without questions about papers.

BEFORE THE FIRST APPOINTMENT

Write down on a sheet: what you feel, since when, what has worsened, current medications, and what you want to ask. If the appointment is in French, write the key sentences in advance or ask someone you trust to come along. Arriving prepared reduces anxiety and makes the appointment count.

IN SHORT

Mild to moderate case: médecin traitant + Mon soutien psy or a Portuguese-speaking psychologist. Persistent or severe case: CMP and psychiatrist. No coverage: PASS and AME. Immediate risk: 3114 or 15, now.

Contact numbers: save them in your phone now

In a crisis, no one searches. People use what is at hand. Save these contacts today, under a clear name, and send this page to the people you love.

3114 · NATIONAL SUICIDE PREVENTION LINE (FRANCE)

3114

Free, 24 hours a day, 7 days a week, from any phone in France. Trained health professionals answer people in distress, as well as family members and friends worried about someone. Service in French. Website: [3114.fr](https://www.3114.fr)

15 · SAMU (MEDICAL EMERGENCY IN FRANCE)

15

For medical emergencies, including psychiatric emergency and immediate risk to life. From a mobile, 112 (the European emergency number) also works and answers in several languages when possible. If the person can move safely, the hospital emergency room also treats.

SOS AMITIÉ · EMOTIONAL LISTENING (FRANCE)

09 72 39 40 50

Anonymous and free listening for anyone who needs to talk, 24 hours a day. Service in French, also by chat at [sos-amitie.com](https://www.sos-amitie.com).

IF YOUR FRENCH FREEZES IN THE MOMENT

Keep your full address written in French in a fixed place (wallet and phone notes): in an emergency call, the address is the first information requested. From a mobile, 112 is the European alternative and usually has resources to assist those who do not speak French.

Support in Portuguese and consular assistance

CVV · CENTRO DE VALORIZAÇÃO DA VIDA (BRAZIL, IN PORTUGUESE)

188 · [cvv.org.br](https://www.cvv.org.br)

The 188 number only works from phones in Brazil, but the chat at [cvv.org.br](https://www.cvv.org.br) serves in Portuguese, including from abroad, according to the hours listed on the website. It is the path to being heard in your own language. The CVV is also the right contact to guide your family in Brazil.

CONSULATE GENERAL OF BRAZIL IN PARIS

gov.br/mre

For serious situations involving Brazilians in France (hospitalization, disappearance, death, vulnerability), the consulate maintains consular assistance channels and an emergency duty line. Check the updated contacts on the official page of the Consulate in Paris on the gov.br portal and keep them saved.

GOLDEN RULE

If you are thinking about hurting yourself, or if someone at your side is at risk right now: 3114 or 15, immediately. Do not leave the person alone. Do not wait to get better on your own, do not wait for Monday, do not wait for it to "pass". Calling is the act of courage, not the opposite.

PRACTICAL SUGGESTION

Save the contacts under names you will find in despair: "HELP France 3114", "HELP emergency 15", "HELP Portuguese CVV". And agree with two people close to you that they do the same.

Specific services: young people, students, accessibility

France maintains channels dedicated to specific groups. If you, your child, or someone in your network fits one of these profiles, the path is shorter than it seems.

114 • EMERGENCY FOR DEAF PEOPLE OR PEOPLE WITH SPEECH DIFFICULTIES

114

National emergency channel for people who are deaf, deafblind, hard of hearing, or aphasic. Works by app, website, chat, video, or SMS. Free, 24 hours a day.

FIL SANTÉ JEUNES • FOR YOUNG PEOPLE AGED 12 TO 25

0800 235 236

Free and anonymous health listening and guidance line for young people, daily from 9 am to 11 pm. Also offers a chat at filsantejeunes.com. A valuable resource for children of immigrants who already speak French.

SANTÉ PSY ÉTUDIANT • FOR HIGHER EDUCATION STUDENTS

santepsy.etudiant.gouv.fr

Public program that gives students 12 free, renewable sessions with a partner psychologist, at no cost. If you or someone in the family studies in France, this is one of the fastest paths, with no financial barrier.

QUICK SITUATION GUIDE

Intense distress or worry about someone: 3114. Immediate danger: 15 or 112. Deaf or aphasic person: 114. Young person aged 12 to 25: Fil Santé Jeunes. Student: Santé Psy Étudiant. Mild to moderate distress: Mon soutien psy or CMP.

Let your family know as much as possible about you

This is the most important tip in this handbook, and the least practiced.

Many immigrants shield their family from their own reality: they hide difficulties so as not to worry anyone, show only the Eiffel Tower and never the winter. The result is perverse: when something goes wrong, no one in Brazil knows where you live, whom to talk to, or where to begin. Distance already separates the bodies; silence separates the rest.

Build a life sheet with your family and keep it up to date:

- Your full address in France, with postal code, and the name on the intercom if it differs from yours.
- The name and phone number of two or three trusted people here: a friend, a colleague, a neighbor, a community leader.
- Where you work or study, with address and contact.
- Your médecin traitant and, if you have one, your psychologist or psychiatrist.
- A copy of your main documents (passport, titre de séjour, carte Vitale).
- Your routine in broad lines: work days, schedules, usual routes.
- France's emergency numbers: 3114, 15, 112, and the consulate's contact.
- A regular contact agreement: a call or message on a fixed day. If you go silent, your family knows something is off pattern and knows whom to call.

And the hardest part: tell the truth about how you are. A family that only receives postcards has no way to help. You protect no one by hiding your pain; you only guarantee it will be faced alone.

Routine and network: protection built in advance

Prevention in mental health is not a motivational phrase: it is structure. The practices below do not replace treatment, but they build the ground that makes the fall less likely and the rescue faster.

Structure your routine before it structures you

- **Light and movement:** the European winter cuts natural light levels. Go outside during the day, even in the cold, and talk to your doctor about vitamin D. Regular physical activity is one of the best documented protective factors in mental health.
- **Sleep:** regular hours, screens out of the bedroom, caffeine within limits. Broken sleep feeds anxiety, and the reverse as well.
- **Language:** every advance in French reduces dependence and daily stress. Treat the French course as an investment in health, not only in career.
- **Finances:** financial instability is one of the immigrant's biggest stressors. A simple spreadsheet and a reserve, even a small one, buy peace of mind.

Build a network before you need it

- Frequent the community's spaces: associations, events, sports groups, churches, courses. A support network is built in normal times, not in crisis.
- Cultivate at least one local friendship, French or international. It opens the country from within.
- Keep bonds with Brazil alive, but in balance: living only on that time zone prevents you from existing here.
- Show up in person, not only online. A WhatsApp group does not replace a coffee.

START SMALL

Do not try to build the ideal life in one week. Choose one item from this page, just one, and practice it for two weeks before adding the next. Emotional structure is built like savings: small, constant deposits.



CHAPTER 07

How to support someone

No crossing needs to be made without support.

How to support someone in the community

In immigration, we are each other's family. Knowing how to act when a compatriot is sinking is a collective responsibility, and there is a method for it.

What to do

- **Ask directly.** "I have noticed you different lately. How are you, really?" Asking about suffering, including about thoughts of death, does not plant the idea: it opens the door the person could not open.
- **Listen without fixing.** Resist the temptation to answer with an immediate solution. First, presence.
- **Take it seriously.** Sentences like "I wish I could disappear" are not drama. Treat them as information, not as exaggeration.
- **Help concretely.** Offer to go along to the doctor, to help call 3114, to watch the children on the day of the appointment, to translate during the visit.
- **Activate the network.** With the person's consent, involve family and close friends. In immediate risk, call 15 even without consent, and do not leave the person alone: life comes before embarrassment.
- **Stay present.** Support is not a single gesture. A message two days later is worth more than a speech on the day.

FOR THOSE WHO CARE

Those who support also need support. If you are holding someone through a crisis, 3114 also answers family members and people close by, and the practices in chapter 06 apply to you.

What to avoid, and a final word

What to avoid

- "You have to be strong." The person already is. That is exactly why they are exhausted.
- "Others have it worse." Comparison does not heal, it only adds guilt.
- "This is lack of faith" or "lack of gratitude for the opportunity". Depression is a health condition, not a moral deficit.
- Promising absolute secrecy in the face of risk to life. The commitment is to the person's safety.
- Diagnosing on your own or recommending medication. That is a professional's role.

A final word

Depression lies. It says you are a burden, that no one would understand, that it will not pass. None of those sentences survives contact with real help: treatment works, severe conditions improve, and the vast majority of people who cross through a crisis look back relieved to have stayed and to have asked for help.

You did not cross an ocean to face this alone.

THE ESSENTIALS OF THIS HANDBOOK, IN SIX LINES

Suffering in immigration is common, documented, and treatable. · More than two weeks of persistent signs: seek professional assessment. · In France: médecin traitant, Mon soutien psy, CMP, PASS, and AME. · In Portuguese: Portuguese-speaking psychologists, online therapy, and the CVV chat. · Immediate risk: 3114 or 15, without waiting. · Let your family know as much as possible about your life here.

It is not about leaving a country. It is about entering another level.

IMPORTANT NOTE

This handbook informs. It does not treat.

The content of this guide is informational and editorial in nature. It does not replace assessment, diagnosis, or treatment by a doctor, psychologist, or other qualified health professional. In the presence of any sign described here, seek professional care. In the presence of immediate risk, in France, call 3114 or 15.

Information on French public services (Mon soutien psy, CMP, PASS, AME, Santé Psy Étudiant) reflects the rules published by official sources at this edition's closing date and may change. Always confirm the conditions in force on ameli.fr and on the French government's channels.

Sources consulted for this edition: Baromètre de Santé publique France (depressive episodes and suicidal behavior, 2017 to 2021); CépiDc/Inserm and DREES, Observatoire national du suicide (mortality, 2020 to 2022); 3114.fr and info.gouv.fr (national prevention line); ameli.fr (Mon soutien psy); cvv.org.br (CVV service); sos-amitie.com (SOS Amitié); filsantejeunes.com (Fil Santé Jeunes); santepsy.etudiant.gouv.fr (Santé Psy Étudiant).



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PLATEFORME ÉDITORIALE INTERNATIONALE

MOBILITÉ · POUVOIR · TRANSFORMATION

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